PathwayAI: From Patient Voices to Community Action

Five-minute live demonstration · Yuwei Zhang · Team GreenAcorn

For the October 22, 2026 showcase; confirm the organizer's schedule. This script reflects the local September 3, October 8 and October 15 office-hours improvements, not verified live deployment. Rehearse navigation and speech together with a timer.

Prepare

Use a fresh session with fictional information. Keep external story AI off for the reliable core path. Have screenshots of the personal card and community pathway as a backup. Do not collect real patient contributions during the demo. Keep the sources page ready for questions.

Fictional story:

> I felt tired after an outdoor walk and would like help preparing for a visit. I paid $180 for medical care and $45 for travel. I missed 3 workdays. My unpaid caregiver spent 8 hours.

0:00–0:30 | Your perspective and purpose

Do: Show the start page.

Say: “I’m Yuwei Zhang from Team GreenAcorn. My background in pathology and my work at NIH and CMS connect clinical experience, research and health policy. PathwayAI brings those perspectives together: helping people describe what illness costs them, and helping communities explore what to do next. The patient remains at the center of the evidence.”

0:30–1:35 | A useful personal record

Do: Click Prepare for care. Paste the fictional story, click Organize My Story with external AI off, choose Past 30 days, and show the reviewed card and download.

Say: “Illness can make it hard to explain what has happened. This fictional person paid 180 dollars for care and 45 dollars for travel, missed three workdays, and received eight hours of unpaid help. The person checks the copied details before using them. The card shows 225 dollars in payments; days and hours stay separate. Caregiver reporting and daily-life effects can also be labeled. The reviewed summary can be downloaded for a visit. The local organizer works without an AI service.”

1:35–2:20 | County context

Do: Choose Explore community burden. Show the New York map, Dutchess–Orange comparison and county cost illustration.

Say: “County reports show disease patterns but not the whole human burden. We bring together public surveillance, federal community context and published cost references. The two-county societal illustration is about 4.11 million dollars in 2016 dollars: reported cases multiplied by a study mean. It illustrates scale; it is not measured local spending or a cost of diagnostic delay. Public CMS aggregates cannot reconstruct an individual's journey before diagnosis.”

2:20–3:35 | Patient voices to community action

Do: Find Patient voices → community action. Show Getting to care, then switch to Unpaid care and daily life. Open See how comparable patient reports could be summarized.

Say: “Office-hours advocates reminded us that the burden includes unpaid care, isolation, lost opportunities, and difficulty reaching benefits—not just bills. Here a community can explore a need, the relevant county context, a possible response, who could help, and what to measure.

“This separate demonstration uses six invented cards. Five report transportation barriers; one answer is missing. We keep residence, reporting period and journey phase comparable. We do not add these cards—or this person's private record—to county case counts. A real consenting participant summary would describe contributors, not every person in the county.

"The decision is specific: should a county care-navigation team investigate a small ride-navigation pilot? Patients and caregivers help decide whether that priority is right. Needs change, so follow-up matters; a nearby clinic alone does not establish usable care."

3:35–4:20 | Transparent assumptions and a usable output

Do: Open Explore an assumed travel-cost scenario; leave three trips at 15 dollars. Show the 45-dollar result. Point to Download patient voice-to-action brief.

Say: “Experts also encouraged useful estimates with transparent assumptions. Here three assumed round trips at 15 dollars gives 45 dollars for one invented person's next 30 days. Change the assumptions and the arithmetic changes. This is not a quote, county demand or a savings claim.

“The downloadable action brief carries the selected priority, proposed response and follow-up measures into a partner conversation. For transport, we could track requested and completed rides, appointments attended and needs still unmet.”

4:20–5:00 | AI, human governance and next step

Do: Point to Ask AI about the public evidence without requiring a live response. Show the brief download or return to the title slide.

Say: “AI can organize a story, but the person reviews its meaning. Data can reveal patterns, but county averages cannot speak for an individual. Models can explore possibilities, but people govern decisions. The patient remains at the center.

“PathwayAI connects that experience to a useful output: a personal summary, a support request, or a community action brief. Next, a secure, consent-based pilot will test whether people reach useful support. That is the bridge I want to build between clinical experience, evidence and policy.”

If the app or AI is unavailable

After ten seconds, switch to the saved fictional example. Say which action did not complete. The maps, local organizer, templates and deterministic demonstrations do not require an OpenAI call. A quota error shows a service notice; admin mode provides API billing links. Do not claim that a failed AI call succeeded.

Boundaries for questions

Real shared collection remains off by default. Secure storage, reviewed consent, duplicate handling, accessible participation and reviewed aggregate release are not implemented by this demonstration. Small-group suppression does not ensure statistical precision or privacy. No county partnership, causal savings, faster diagnosis or improved health has been demonstrated. Source-table verification gaps remain documented.